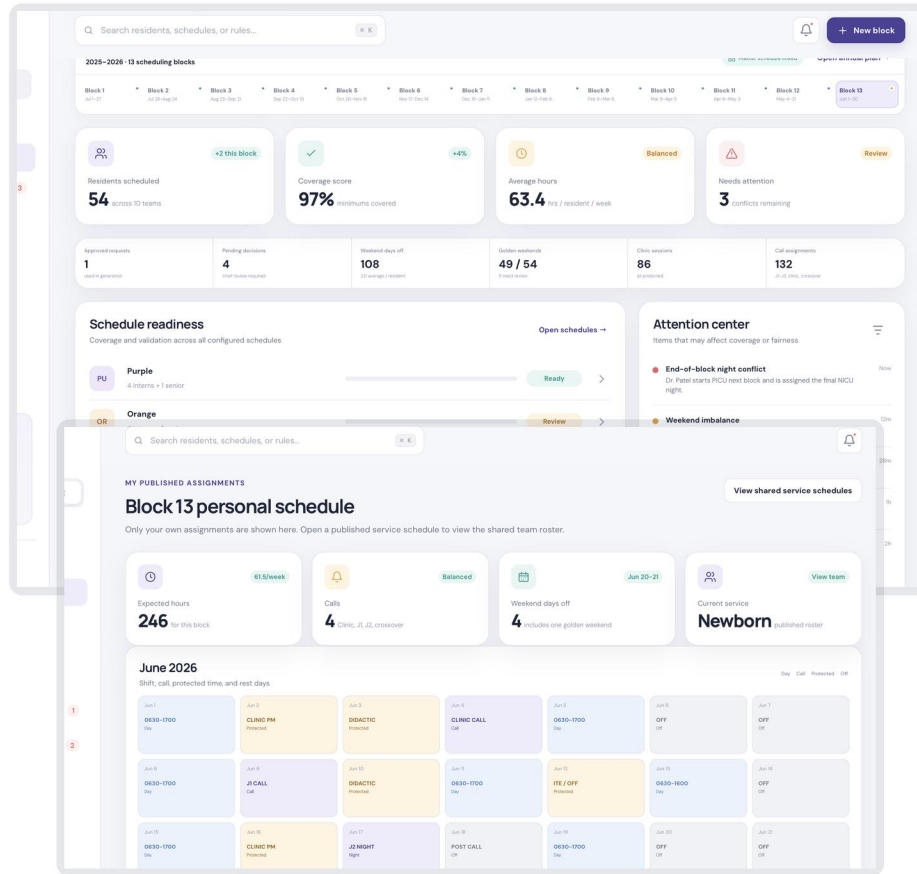


Clarity Schedule

A connected platform for resident requests, annual master schedules, monthly service schedules, approvals, fairness, and publication.

The central idea

Enter program rules and resident data once. Reuse them to generate safe, editable templates for every block and every service.



Less manual reconciliation. Clearer decisions. Safer coverage.

All names and examples in this proposal are fictional demonstration data.

THE PROBLEM WE ARE SOLVING

Scheduling is a chain of manual investigations

A single change can require three people to open several files, compare calendars, check rules, and communicate the same facts repeatedly.

Requests live elsewhere

Vacation, weekends, PTO, conferences, exams, fellowship timing, and special circumstances arrive through different channels.

Rules are reconstructed

Clinic and didactic patterns, outside-rotator rules, service minimums, and call restrictions are checked again every block.

Master and monthly schedules drift

Annual rotation assignments do not automatically populate the residents who need each monthly service schedule.

Coverage is difficult to see

It is easy to oversaturate one service while leaving another below its minimum intern or senior requirement.

Fairness is calculated late

Weekends, golden weekends, calls, nights, clinic sessions, and hours often require separate manual counting.

Changes create new conflicts

A call switch or master-schedule edit may produce clinic, post-call, adjacent-call, or next-block conflicts.

What the current process often requires

1. Search

Find the resident's master schedule, service PDF, clinic pattern, leave, and prior calls.

2. Compare

Check the date against staffing minimums, call rules, post-call needs, and other requests.

3. Communicate

The offering resident, receiving resident, and chief each repeat the same review.

4. Rebuild

Update documents and recount coverage, nights, weekends, and hours after the change.

Clarity turns this investigation into a guided workflow.

The platform pulls the relevant data automatically, displays why a request is eligible or conflicting, and gives chiefs the final decision.

THE SOLUTION

One connected data model powers every schedule

Resident facts, approved requests, institutional patterns, service rules, and the annual master schedule should not be separate islands.

Resident profiles

PGY level, institution, recurring clinic, didactics, eligibility, fellowship plans, and special circumstances.

Approved requests

Vacation, PTO, sick leave, weekends, days off, conferences, exams, and call restrictions.

Institution rules

Outside-rotator patterns, protected educational time, travel buffers, and eligible roles.

Program structure

Blocks, services, units, staffing minimums, shift templates, work hours, and safety rules.

CLARITY SCHEDULING ENGINE

Pull eligible residents from the master schedule, protect fixed activities, propose balanced assignments, and flag conflicts before publication.

Editable master schedule

Capacity-aware annual rotation planning

Service templates

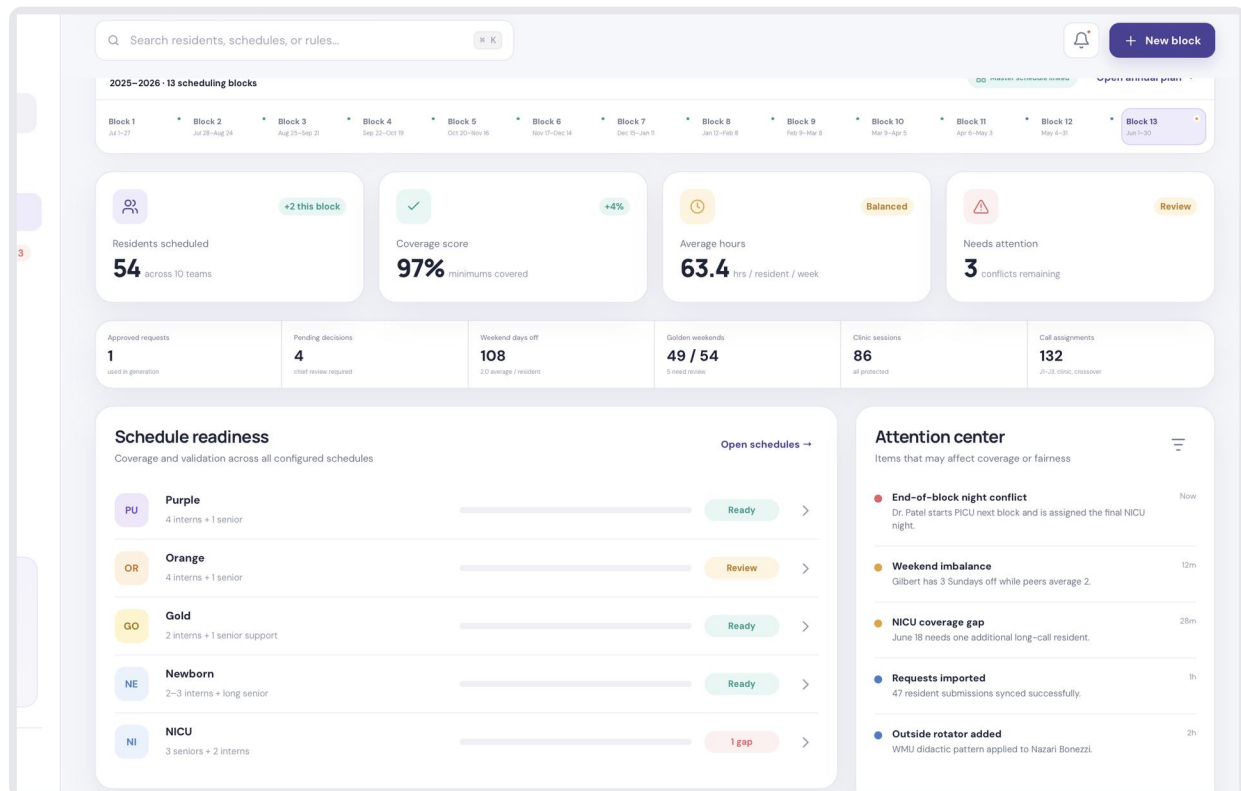
One monthly schedule per service per block

Resident portal

Personal schedule, requests, PTO, and switches

Chief dashboard

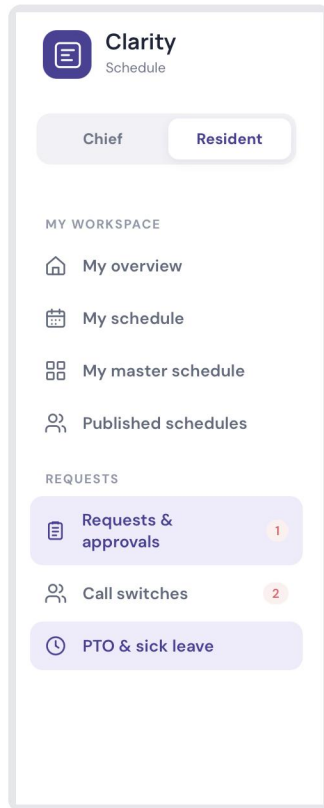
Approvals, readiness, conflicts, and analytics



The result is a block-level view of which service schedules are ready, pending, or need attention.

Everything a resident needs, without exposing private peer data

Residents see their own annual plan and personal assignments, while published service schedules remain shared and read-only.



1 My overview

A summary of current rotation, expected hours, calls, golden weekends, days off, clinic, didactics, and upcoming events.

2 My schedule

A monthly personal calendar showing exact work hours, call type, clinic, protected didactics, exams, post-call time, and rest days.

3 My master schedule

A private block-by-block annual rotation plan. Other residents cannot see it; authorized chiefs can edit the program-wide plan.

4 Published schedules

Read-only service rosters show who is covering each service, where they are assigned, and the hours they work.

5 Requests, call switches, and leave

Structured requests replace emails and messages. Status and chief decisions remain visible to the resident.

Resident privacy by design

A resident can view their own master schedule and personal assignments, but not another resident's private annual plan. Shared service schedules are visible only after chiefs publish them.

Why this matters

Residents spend less time asking where to find schedules or whether a request was received. Chiefs spend less time answering repetitive questions.

My schedule and my private master schedule

Two different views answer two different questions: What am I working this month? Where am I rotating across the year?

Q Search residents, schedules, or rules...

🔔

MY PUBLISHED ASSIGNMENTS

View shared service schedules

Block 13 personal schedule

Only your own assignments are shown here. Open a published service schedule to view the shared team roster.

🕒

61.5/week

🔔

Balanced

📅

Jun 20-21

👥

View team

Expected hours

246 for this block

Calls

4 Clinic, J1, J2, crossover

Weekend days off

4 includes one golden weekend

Current service

Newborn published roster

June 2026

Shift, call, protected time, and rest days

Day Call Protected Off

Jun 1 0630-1700 Day	Jun 2 CLINIC PM Protected	Jun 3 DIDACTIC Protected	Jun 4 CLINIC CALL Call	Jun 5 0630-1700 Day	Jun 6 OFF Off	Jun 7 OFF Off
Jun 8 0630-1700 Day	Jun 9 J1 CALL Call	Jun 10 DIDACTIC Protected	Jun 11 0630-1700 Day	Jun 12 ITE / OFF Protected	Jun 13 0630-1800 Day	Jun 14 OFF Off
Jun 15 0630-1700 Day	Jun 16 CLINIC PM Protected	Jun 17 J2 NIGHT Night	Jun 18 POST CALL Off	Jun 19 0630-1700 Day	Jun 20 OFF Off	Jun 21 OFF Off

My schedule: daily work hours, calls, clinic, didactics, post-call time, and rest days.

Personal workload summary

Expected hours, call count, weekend days off, golden weekends, and current service are calculated from the published assignments.

Protected activities are visible

Clinic, didactics, ITE, conferences, and approved leave are clearly separated from regular service shifts.

Q Search residents, schedules, or rules...

🔔

PRIVATE ANNUAL PLAN

✓ Private

My master schedule

This page is visible only to you and authorized chiefs. Other residents cannot view it.

Block 1
Floor
Jul 1-27

Block 2
NICU
Jul 28-Aug 24

Block 3
Elective
Aug 25-Sep 21

Block 4
PICU
Sep 22-Oct 19

Block 5
ED
Oct 20-Nov 16

Block 6
Newborn
Nov 17-Dec 14

Block 7
Elective
Dec 15-Jan 11

Block 8
Floor
Jan 12-Feb 8

Block 9
Heme/Onc
Feb 9-Mar 8

Block 10
Vacation
Mar 9-Apr 5
Vacation eligible

Submitted annual preferences

Submitted

Chiefs can review and approve these before building the annual schedule.

Fellowship planning
Yes - early electives requested

Top vacation block
Block 10

Preferred first rotation
Floor

Preference result
12 of 20 priorities matched

My master schedule: the resident's private annual rotation plan and approved annual preferences.

The link between annual and monthly scheduling

When the master schedule assigns a resident to NICU in Block 2, the Block 2 NICU schedule automatically begins with that resident in its eligible roster. Chiefs do not need to re-enter the resident manually.

Replace manual eligibility searches with automatic checks

The most time-consuming requests are often the ones that cross several schedules, such as a call switch.

Before Clarity

The offering resident, receiving resident, and chief each inspect call PDFs, clinic schedules, adjacent calls, and post-call implications. Messages move back and forth until everyone agrees.

With Clarity

The resident offers a call, the receiving resident sees immediate eligibility results, and the chief receives one approval item with the evidence already attached.

The screenshot shows a mobile app interface for a 'RESIDENT CALL EXCHANGE'. At the top is a search bar labeled 'Search residents, schedules, or rules...'. Below it is the 'Call switch center' section, which includes a sub-header 'Offer an assigned call, review available calls, and see automatic clinic and back-to-back call checks before involving the chiefs.' and a green status bar 'Schedules checked'. A vertical flow of three steps is shown: 1. Resident offers (Offer an assigned call and acceptable alternatives), 2. Another resident volunteers (Receive a scheduled call, request a swap, and check), and 3. Chief approves (The published schedule changes only after final approval). Below this is the 'Offer one of my calls' section, which includes a sub-header 'Only your assigned clinic 10 calls are listed.' and a 'Private draft' button. The form contains fields for 'Clinic offer' (set to 'Clinic Call - Jan 4 - 17:00-22:00'), 'Preferred exchange date' (06/30/2026), and 'Second option' (06/27/2026). A note for residents and chiefs states: 'Example: I can exchange for another weekend call.' At the bottom, a large blue button with a white plus sign is labeled 'Publish call offer'. A note above the button states 'Eligibility is personalized to Resident A.'

1 Offer an assigned call

The system lists the resident's actual call assignments and keeps the original assignment active until approval.

2 State acceptable alternatives

Residents can identify dates or call types they can exchange, reducing open-ended messaging.

3 Screen the receiving resident

The app checks clinic on the call day, clinic the next day, an existing call on the same date, and calls on adjacent days.

4 Send one complete request

The chief sees the offering resident, receiving resident, date, call type, eligibility results, and approval controls together.

5 Document the final decision

Approval or decline is visible to both residents and can update the schedule without losing the review trail.

Safety rule example

A resident cannot accept a call on the same day as clinic, the day before clinic, or next to another call. The reason is shown immediately.

Build reusable templates for every service

Each block may require many separate service schedules. Every service can have different staffing, hours, units, calls, nights, and fairness targets.

Service builder: define the program structure and service-specific coverage and fairness rules.

Add any service

NICU, PICU, Heme/Onc, cardiology, colored floor teams, night senior, jeopardy, or a custom service.

Define units and locations

Divide one service into Unit 1, Unit 2, floor teams, consult roles, procedure coverage, or named responsibilities.

Set staffing minimums

Specify required interns, seniors, PGY eligibility, outside rotators, and supplemental call-pool residents.

Create shift templates

Standard day, night, short call, long call, procedure call, weekend, post-call, protected time, or custom tasks.

Configure once, reuse across every block

The chief can still edit any generated assignment. Templates save setup time without replacing clinical judgment.

Start with a master-linked draft, then edit it like a spreadsheet

The platform pulls residents assigned to the selected service and block, applies protected activities, proposes shifts, and continuously recalculates coverage and fairness.

Clarity Schedule

Chief Resident

WORKSPACE

- Overview
- Schedule builder
- Schedules
- Analytics
- Master schedule
- PEOPLE & RULES
- Residents & approvals
- Institution rules

Search residents, schedules, or rules...

BLOCK 1 · JUL 1-27, 2026

Schedule workspace

ACADEMIC YEAR 2025-2026 - 13 scheduling blocks

Purple Orange Gold Newborn NICU PICU Home/Onc Jeopardy Night Senior Green Team

Filter Week 1 - Jun 1-7 Today + Add resident

Purple - Block 1 master-linked roster

Coverage plan 5-5 4-6 4/5 6 stretches

Resident RA RK RD RE RF

MON Jun 1 TUE Jun 2 WED Jun 3 THU Jun 4 FRI Jun 5 SAT Jun 6 SUN Jun 7

17:00-07:00 17:00-07:00 17:00-07:00 17:00-07:00 17:00-07:00 POST CALL OFF

06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 18:15-07:00 18:15-07:00

06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-16:00 06:30-19:00

06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-19:00 OFF

06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 06:30-17:00 OFF OFF

Daily coverage 5/5 5/5 3/5 4/5 5/5 3/5 2/5

Schedule insights

Balanced four-week draft 94

Coverage

Fairness

Schedule workspace: resident rows, daily assignments, coverage totals, live warnings, and fairness indicators.

Master-linked roster

The initial resident list comes from the annual rotation plan. Eligible call-pool residents can be added when needed.

Drag-and-edit flexibility

Chiefs can move assignments, change residents, copy shifts, adjust times, reorder rows, and override the generated draft.

Coverage guardrails

Daily totals reveal understaffed dates and whether required interns, seniors, units, and call roles are filled.

Fairness visibility

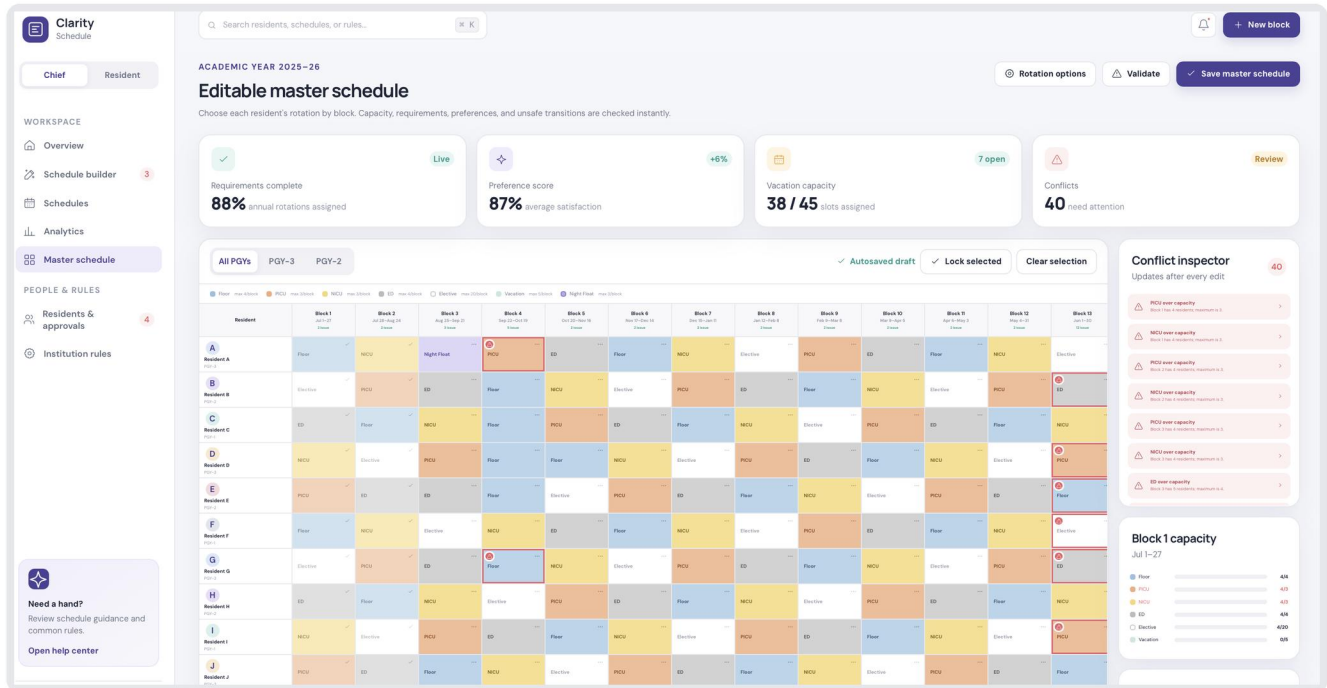
Night stretches, weekend days, golden weekends, calls, and hours can be compared before publication.

Expected result

A publish-ready service schedule that lists resident names, exact work hours, coverage responsibility, calls, protected time, and rest days.

Edit the master schedule while protecting service capacity

Resident preferences matter, but every block must still contain enough residents for required services and clearly identify where electives, vacation, and outside rotators are eligible.



Editable annual master schedule: each resident by block, with capacity warnings and a live conflict inspector.

Resident preference matching

Use annual rankings, requested vacation blocks, fellowship timing, life events, and preferred sequence as decision support.

Block capacity checks

See immediately when PICU, NICU, floor, ED, or another rotation is over capacity or below its minimum.

Eligible elective and vacation blocks

Clearly distinguish flexible blocks from mandatory inpatient assignments and account for outside rotator eligibility.

Unsafe transition alerts

Flag end-of-block night assignments followed by inpatient rotations or other transitions that prevent appropriate recovery.

Goal: honor preferences without creating a short team

Every edit is evaluated against rotation capacity and downstream monthly schedule requirements.

Approve requests once; apply institution rules automatically

Chief decisions should become scheduling data, and recurring institutional rules should not require resident-by-resident re-entry.

PEOPLE & RULES

- Residents & approvals (4)
- Institution rules

Recurring didactics
Inherited from Corewell Health

Friday 12:30-1:00
Protected activity: Newswatching with

Block 1 exceptions
Leave, exams, conferences, and one-time days off

ITE examination
June 10, 10:00-12:00

Call switch approvals
The app has already checked both residents' calls and clinic dates. Chiefs retain the final approval.

Requests awaiting chief decision
Only approved requests are used by the schedule generator. Suggestions are advisory; chiefs retain the final decision.

Chief approval queue: call switches and resident requests include conflicts, eligibility checks, and decision controls.

Approve or decline requests

Only approved requests influence generation. Declined requests remain documented but do not constrain the schedule.

Immediate downstream effect

Approved leave, exams, weekends, and switches can update availability, protected time, and schedule validation automatically.

Clarity Schedule

Search residents, schedules, or rules...

SCHEDULING CONFIGURATION

Rules and institutions

Build reusable institutional patterns and customize the requirements for every schedule tab.

Academic year: 2025-2026 · 13 scheduling blocks

Teams & tabs | Shift templates | **Institution profiles** | Service requirements | Global safety rules

Institutions

- CH Corewell Health (33 listed residents)
- WS Wayne State / DMC (3 listed residents)
- WMU Western Michigan (3 listed residents)
- HFM Henry Ford (5 listed residents)

INSTITUTION PROFILE
Corewell Health
Primary residency program. Standard inpatient and call eligibility rules apply.

Automatic application
These recurring rules are applied whenever the master schedule assigns a resident from Corewell Health to a monthly service.

Recurring protected time
Core and service patterns repeat across all 13 blocks unless a block override is added.

ACTIVITY	APPLIES TO	RECURRING	TIME	SCHEDULING EFFECT
Continuity clinic	Resident-specific	Weekly assigned day	Resident-specific: weekly half day	Protected: no call overlap
Institution didactics	All rotators	Weekly	Friday 12:30-1:00	Protected: travel buffer
Conference / attendance	Selected residents	Uploaded by block	Varies	Full or partial day off

Institution eligibility
Control which service roles this institution's residents may fill.

Purple / Orange Intern (English when assigned) ☒ Gold Intern (English when assigned) ☒ Newborn Intern (English when assigned) ☒ NICU day (English when assigned) ☒

Institution profiles: recurring clinic, didactics, conferences, eligibility, and scheduling effects are reused automatically.

Different institutions, different patterns

Main-program residents and outside rotators can follow different clinic days, didactics, travel buffers, and eligible service roles.

Editable and overrideable

Chiefs can edit a reusable institutional profile or add a block-specific exception for an individual resident.

DEPARTMENT IMPACT

What changes when the schedules are connected

The value is not only faster schedule creation. It is better visibility, safer changes, clearer accountability, and a reusable operational process.

Chiefs

TIME AND EFFORT SAVED

Fewer manual cross-checks and fewer repeated data-entry tasks.

RESULT

More time for education, resident support, and complex judgment.

Residents

TIME AND EFFORT SAVED

Clear personal schedules, private annual plans, structured requests, and safer call switches.

RESULT

More trust, predictability, and transparency about workload and decisions.

Department

TIME AND EFFORT SAVED

Consistent rules, documented approvals, live coverage readiness, and linked analytics.

RESULT

Better resilience, auditability, continuity, and awareness of staffing risk.

A practical evaluation plan

Phase 1

Configure one academic year, sample resident profiles, institution rules, and two services.

Phase 2

Generate one block in parallel with the current process and record editing time and conflicts.

Phase 3

Compare coverage, fairness, usability, and chief/resident feedback before wider adoption.

The proposed outcome: a transparent, reusable, and safer scheduling workflow that reduces administrative effort without removing chief control.

All resident names and scheduling records shown are fictional demonstration data.